

Supplementary Appendix

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This appendix has been provided by the authors to give readers additional information about the work.

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Supplementary Methods

Investigator's choice dosing schedule

Patients in the daratumumab combined with pomalidomide and dexamethasone (DPd) group received DPd in 28-day cycles (see also **Table S2**). In each cycle, patients received subcutaneous daratumumab (1800 mg weekly in cycles 1 and 2; every 2 weeks in cycles 3 to 6; every 4 weeks in cycle 7+), oral pomalidomide at a dose of 4 mg on days 1 through 21 of each cycle, and oral/intravenous dexamethasone at a dose of 40 mg (if ≤ 75 years of age) or 20 mg (>75 years of age or body mass index of <18.5) weekly of each cycle. Patients in the daratumumab, bortezomib and dexamethasone (DVd) group received DVd in 21-day cycles for 8 cycles, followed by 28-day cycles for cycles 9+. Patients received subcutaneous daratumumab (1800 mg weekly in cycles 1 to 3; every 3 weeks in cycles 4 to 8; every 3 weeks in cycle 9+), subcutaneous bortezomib on days 1, 4, 8, and 11 of cycles 1 to 8 only, and oral/intravenous dexamethasone at a dose of 20 mg on days 1, 2, 4, 5, 8, 9, 11, and 12 of cycles 1 to 8, only, except for patients ≥ 75 years of age of with a body mass index of <18.5 , who received 20 mg of dexamethasone weekly only on days 1 and 8 for first 8 cycles.

Endpoint definitions

Progression-free survival (PFS; primary endpoint): the time from the date of randomization to the date of first documented disease progression, as defined by International Myeloma Working Group (IMWG) criteria,¹⁻³ or death due to any cause, whichever occurs first. Disease progression was assessed by an Independent Review Committee, investigator, and validated computerized algorithm; however, assessment by the Independent Review Committee was considered the primary method.

Minimal residual disease (MRD) negativity: defined as patients who achieved MRD negativity at a threshold of 10^{-5} (ie, at or below 10^{-5}) at any timepoint after the date of randomization and before disease progression or start of subsequent antimyeloma therapy. MRD negative $\geq$ CR rate: patients achieving MRD negative status within 3 months prior to achieving CR or better response or at any time after CR or better response and prior to progressive disease or subsequent antimyeloma therapy. The next-generation sequencing (NGS) primary analysis set was used for MRD analyses and included all randomized patients in the study except for those recruited in China. NGS (ClonoSEQ, Adaptive Biotechnologies) was the method of MRD assessment in all study countries other than China in which next-generation flow was used for patients enrolled.

Overall response: defined as patients who achieve partial response, very good partial response, complete response, or stringent complete response, according to IMWG criteria¹⁻³ and based on Independent Review Committee assessment.

Complete response or better: defined as patients who achieve complete response or stringent complete response according to IMWG criteria¹⁻³ and based on Independent Review Committee assessment.

Very good partial response or better: defined as patients achieving a very good partial response, a complete response, or stringent complete response, according to the IMWG criteria¹⁻³ and based on Independent Review Committee assessment.

Duration of response: calculated among responders (patients with a partial response or better response) from the date of initial documentation of a response to the date of first documented evidence of progressive disease according to IMWG response criteria¹⁻³ and based on Independent Review Committee assessment, or death due to any cause, whichever occurred first. Patients who had not progressed and were alive were to be censored at the last disease evaluation before the start of subsequent antimyeloma therapy.

Overall survival (OS): measured from the date of randomization to the date of death. If the patient had unknown vital status, data was censored at the last known date they were alive.

Multiple Myeloma Symptom and Impact Questionnaire (MySIm-Q): patients completed the MySIm-Q, a disease-specific patient reported outcomes assessment with established content validity for patients with multiple myeloma, composed of 17 items resulting in a symptom subscale and an impact subscale. Scores were summarized descriptively Time to worsening of symptoms was defined as the interval from the date of randomization to the start date of worsening on the MySIm-Q total symptom score. Death due to disease progression was considered as worsening. The anchor-based worsening minimally importance difference on the MySIm-Q symptom scale was the mean change on the MySIm-Q symptom scale associated with a 2-point increase on the Patient Global Impression of Severity symptom question between baseline and C4D1. Worsening was defined as a score greater than or equal to the minimally importance difference threshold without subsequent improvement to a score below this level. Refer to the statistical analysis plan for additional details.

Adverse events: All adverse events (note: disease progression was not a reportable adverse event) and special reporting situations, whether serious or nonserious, were reported from the time a signed and dated informed consent form was obtained until 30 days after the last dose of study treatment or until the start of subsequent antimyeloma therapy, if earlier, which may have included contact for follow-up of safety. Any adverse events that occurred more than 30 days after the last dose of study treatment and considered related to study treatment were reported to the sponsor. Safety was assessed by adverse events, laboratory test results, vital sign measurements, physical examination findings, Eastern Cooperative Oncology Group performance status grade, and immune effector cell-associated encephalopathy score. The verbatim terms used in the eCRF by investigators to identify adverse events were coded using the Medical Dictionary for Regulatory Activities. Any adverse event that occurred at or after the start of administration of study treatment through the day of last dose +30 days, or the day prior to start of subsequent anti-myeloma therapy, whichever occurred earlier, was considered treatment-emergent and was included in the analysis. For each adverse event, the percentage of patients who experienced ≥ 1 occurrence of the given event was summarized by treatment group.

All endpoints explored in the MajesTEC-3 study are defined in the Protocol and Statistical Analysis Plan, which is available with the full-text article at NEJM.org.

Timing of assessments for disease progression and minimal residual disease

Disease evaluations (as specified fully in the protocol Schedule of Activities), were performed by a central laboratory during the treatment phase for all patients every 28 days (± 7 days), except in cycles 1-8 for patients in the DVd group, where every 21 days (± 7 days) is specified, even if the

start of a new cycle was delayed. For MRD assessments in both treatment arms, bone marrow aspirate samples were collected at screening to evaluate baseline clonality of bone marrow plasma cells, and then at time of suspected complete response or stringent complete response (complete response or better).

Additional statistical methods

Hypothesis testing

The primary hypothesis was to be tested at the 0.05 significance level (overall). The exact significance level for superiority at the PFS primary analysis was determined by the observed number of events using the O'Brien-Fleming boundaries as implemented by the Lan-DeMets alpha spending method. Based on 231 PFS events observed at the interim analysis, the two-sided alpha to be spent in the interim analysis was 0.0139. If the observed two-sided p-value was smaller than 0.0139, the superiority of Tec-Dara over DPd/DVd with respect to PFS would be established. If the primary endpoint of PFS was statistically significant, then the major secondary endpoints (complete response or better, overall response, MRD negativity, OS, and time to worsening of symptoms in the MySIm-Q total symptom score) would be sequentially tested for superiority, each with an overall 2-sided significance level of 0.05, by using a hierarchical procedure proposed by Tang and Geller⁴ to strongly control familywise type I error. For overall survival, group sequential testing was performed; the first interim analysis was performed at the same time as the PFS interim analysis, spending a nominal alpha of 0.0001 (2-sided).

Handling of missing efficacy/safety data

No imputation was planned for any missing efficacy endpoint values or missing safety evaluations; all available data were presented.

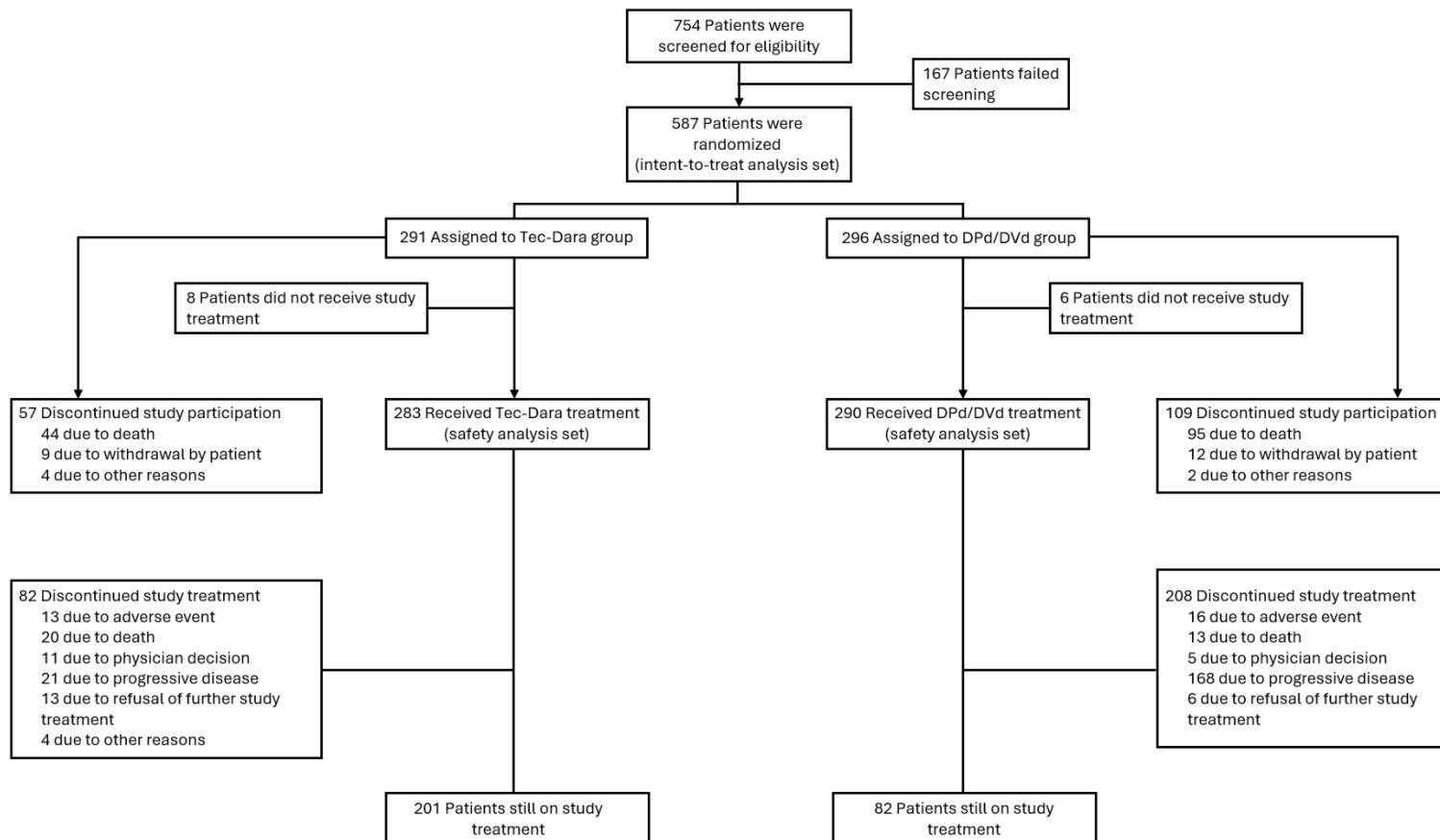
Assessment of proportional hazards assumption of the Cox regression models

For time-to-event endpoints, the proportional hazards assumption of the Cox regression models was assessed by visual inspection of the Kaplan-Meier (KM) curves. If there was an evident cross of the KM curves, a Restricted Mean Survival Time (RMST) analysis was performed to assess the robustness of the results of the primary analysis.

Handling of intercurrent events and censoring rules in PFS and OS analyses

Treatment switching to other anticancer therapy is an intercurrent event in both PFS and OS analyses. For PFS analysis, this intercurrent event was handled following the ‘hypothetical strategy’; that is, PFS was censored if occurring after the start of subsequent therapy. For OS analysis, the ‘treatment policy strategy’ was followed: time to death was used for those patients who die, irrespective of switching to subsequent anti-myeloma therapy or not. In addition, in the PFS analysis, patients who had a PFS event immediately after two or more consecutive missed disease assessments, were censored (refer to the statistical analysis plan for more details on event/censoring rules). The difference in censoring rules/handling of intercurrent events between PFS and OS analyses could give rise to discrepancies in results between the two analyses.

Figure S1. CONSORT Diagram for MajesTEC-3 Study.



Eligible patients were randomized to receive teclistamab in combination with daratumumab (Tec-Dara group), or investigator's choice of daratumumab in combination with pomalidomide and dexamethasone or daratumumab in combination with bortezomib and dexamethasone (DPd/DVd group).

Figure S2. Duration of Response.

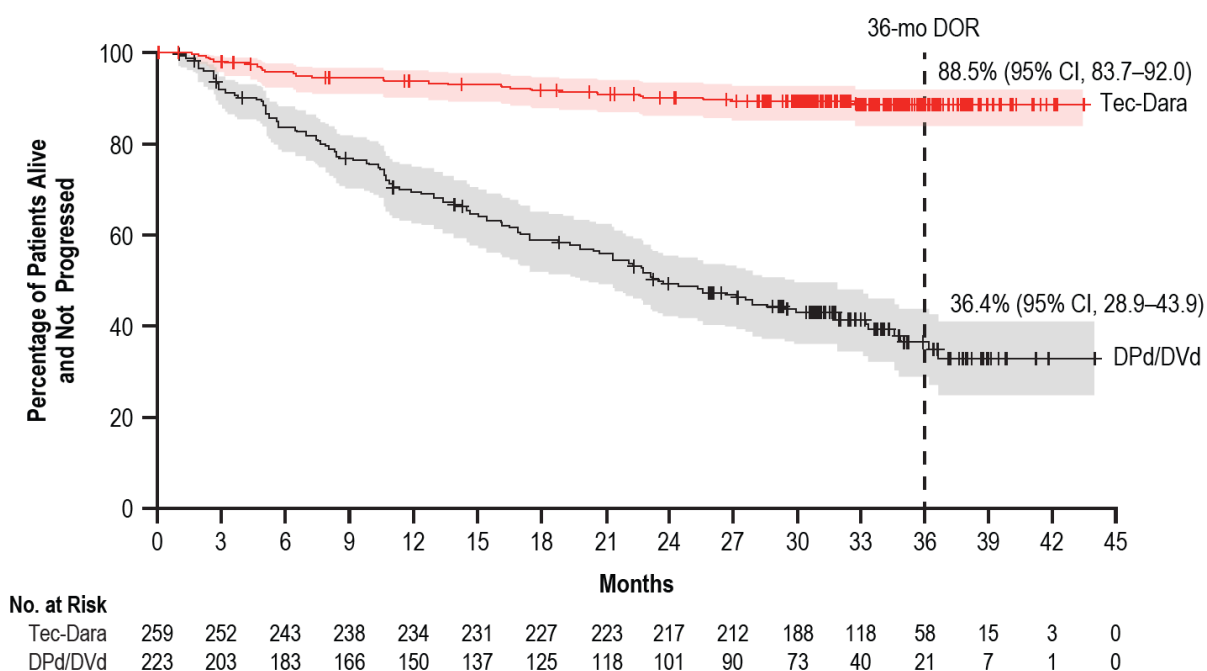


Figure shows Kaplan-Meier estimates of duration of response (DOR) among responding (partial response or better) patients who were randomized to receive teclistamab in combination with daratumumab (Tec-Dara), or investigator's choice of daratumumab in combination with dexamethasone and either pomalidomide or bortezomib (DPd/DVd). Red shading indicates the 95% CI for the Tec-Dara group and grey shading the 95% CI for the DPd/DVd group. Tick marks indicate censored data. Responses/progressive disease were assessed by an Independent Review Committee per International Myeloma Working Group response criteria. Note, CI widths have not been adjusted for multiplicity and may not be used in place of hypothesis testing. CI denotes confidence interval.

Figure S3. Minimal Residual Disease Negative Complete Response or Better Rates at the 10^{-5} Threshold Based on the Next-Generation Sequencing Primary and Evaluable Analysis Sets.

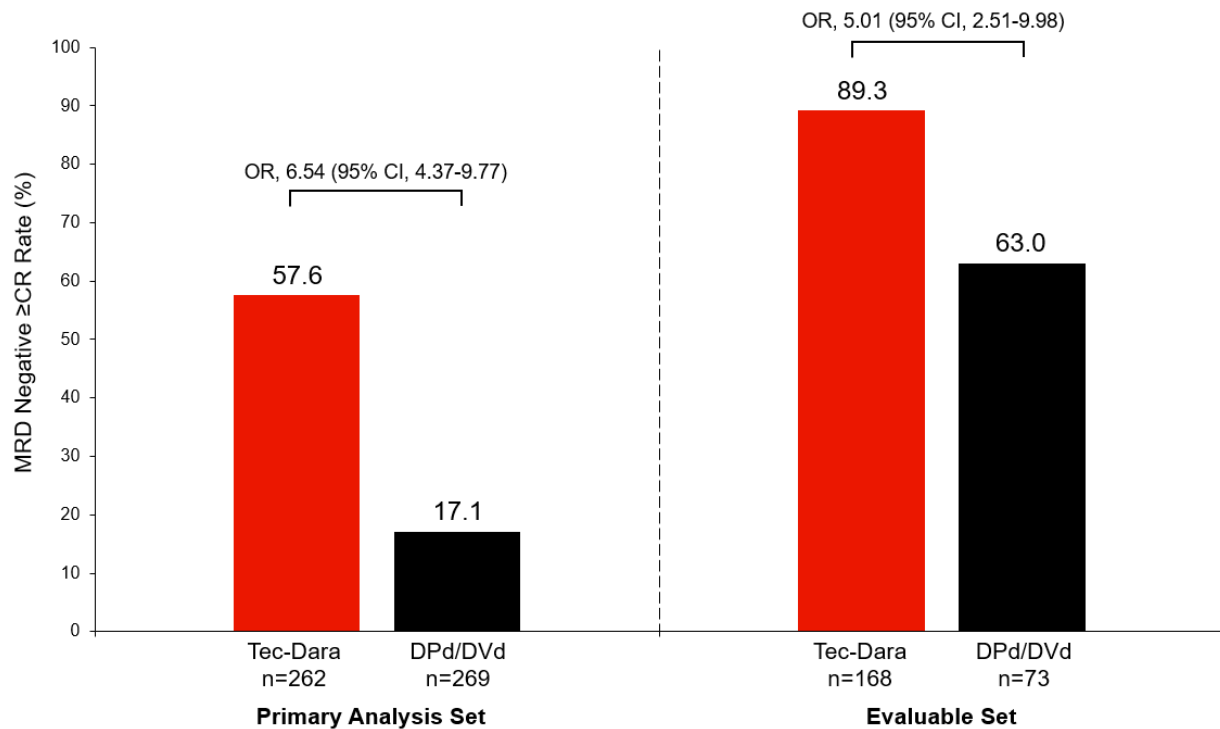


Figure shows minimal residual disease (MRD) negative (10^{-5}) complete response or better ($\geq$ CR) rate, which refers to MRD negativity achieved within 3 months prior to achieving a complete response or stringent complete response, or at any time after a complete response or stringent complete response and before progression or subsequent therapy. 10^{-5} refers to MRD negativity rates at or below 10^{-5} . The MRD next-generation sequencing (NGS) primary analysis set included all randomized patients in the study except patients recruited in China, due to their MRD evaluation with a next-generation flow cytometry test. The MRD NGS-evaluable set (post-hoc analysis set) included all patients with a successful baseline calibration and ≥ 1 post-baseline MRD sample at or below 10^{-5} by NGS with a positive or negative result. Note, confidence interval widths have not been adjusted for multiplicity and may not be used in place of

hypothesis testing. Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, OR odds ratio, CI confidence interval.

Figure S4. Fatal Infection Incidence in the MajesTEC-3 Study Before and After Implementation of Reinforced IgRT Guidance Overlaid with COVID-19 Pandemic Worldwide Deaths.

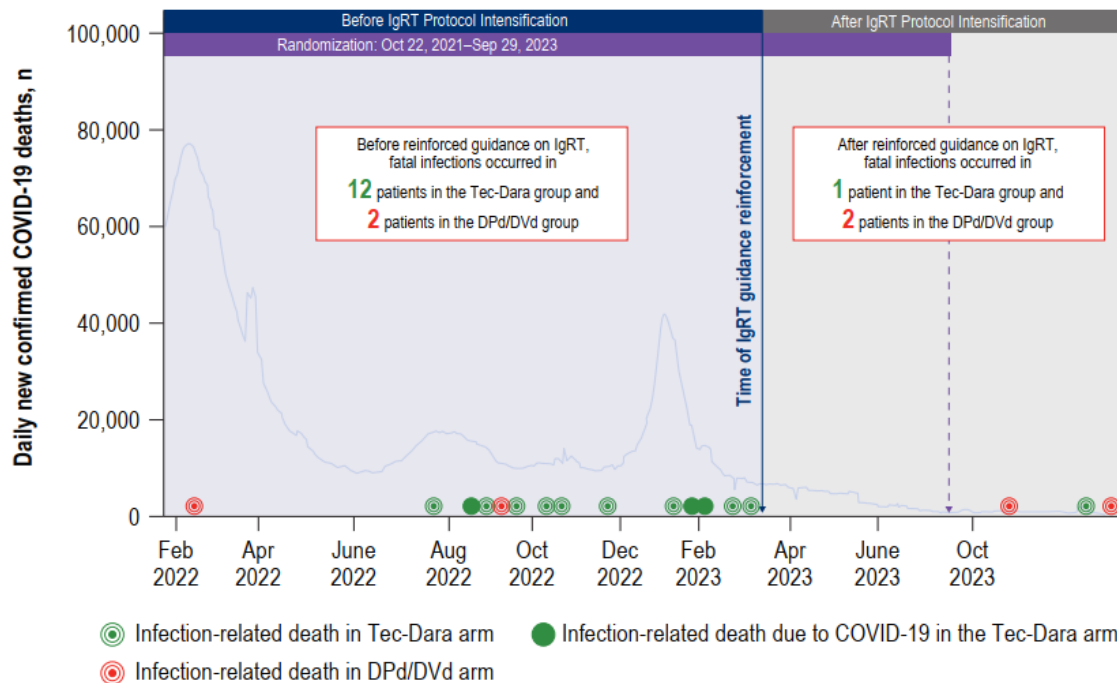


Figure shows daily new confirmed worldwide deaths due to COVID-19, as reported by the World Health Organization,⁵ and occurrence of fatal infections in the teclistamab in combination with daratumumab (Tec-Dara) and daratumumab in combination with dexamethasone and either pomalidomide or bortezomib (DPd/DVd) groups of the MajesTEC-3 study. IgRT denotes immunoglobulin replacement therapy. Three patients in the Tec-Dara group who died before the implementation of reinforced IgRT guidance had ≥ 1 dose of immunoglobulin treatment.

Figure S5. Mean (Standard Deviation) Serum Teclistamab Predose Concentrations

($\mu\text{g/ml}$).*

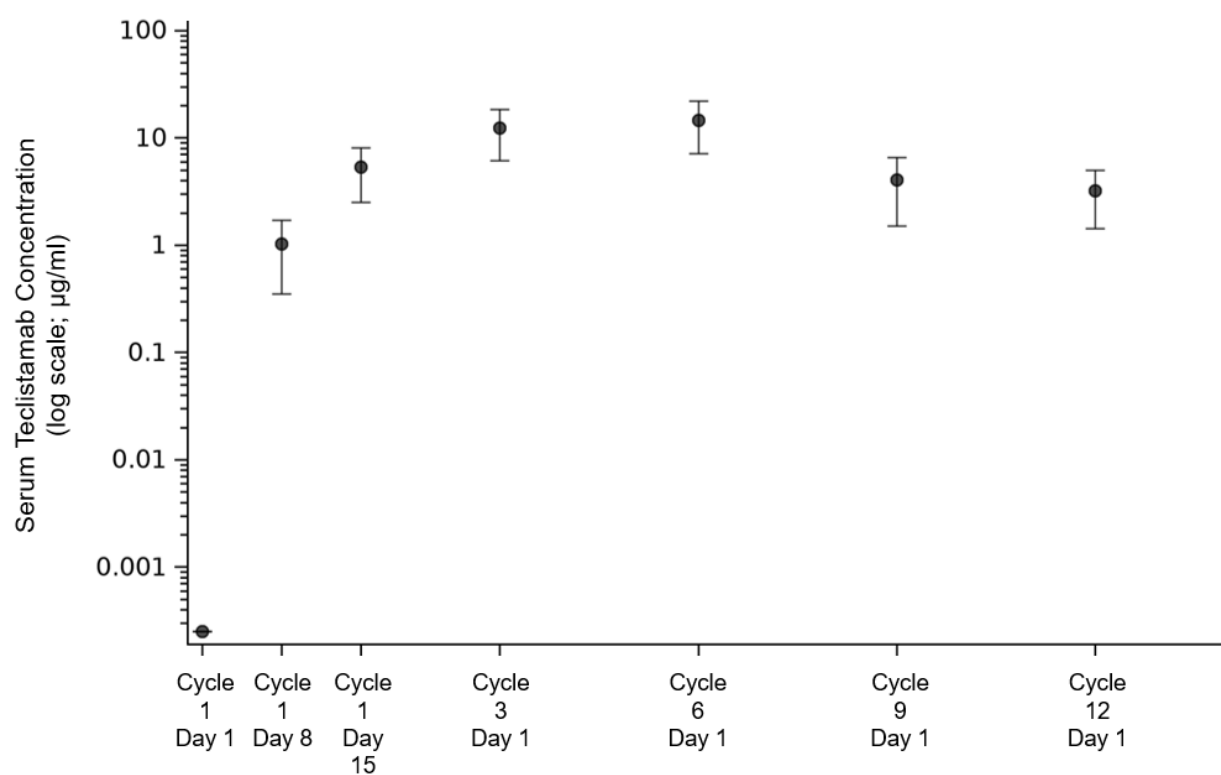
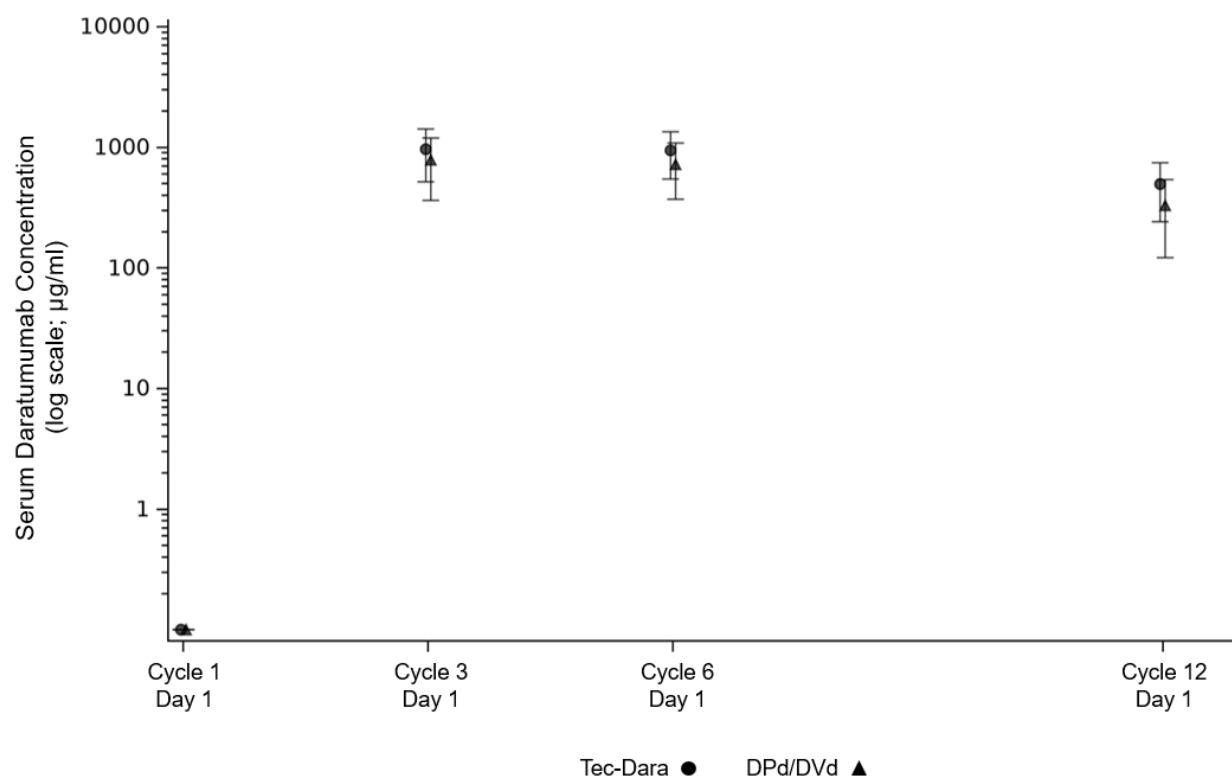


Figure shows predose concentrations of teclistamab in the Tec-Dara treatment group. At each timepoint, concentration values are plotted on the log scale and the error bars are mean $\pm$ standard deviation.

Figure S6. Mean (Standard Deviation) Serum Daratumumab Predose Concentrations (µg/ml).*



Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib. Figure shows predose concentrations of daratumumab in the Tec-Dara and DPd/DVd treatment groups. At each timepoint, concentration values are plotted on the log scale and the error bars are mean $\pm$ standard deviation.

Table S1. Summary of Dose Schedule for Tec-Dara Group.

Study Drug	Notes	Cycle 1					Cycle 2		Cycle 3-6		Cycle 7+
		Day					Day		Day		Day
		1	2	4	8	15, 22	1	8, 15, 22	1	15	1
Teclistamab step-up dose	- Step-up dose 1 must have been administered ≥ 20 hours after daratumumab SC given on C1D1 - Thereafter, there must have been ≥ 2 days between step-up doses		0.06 mg/kg	0.3 mg/kg							
Teclistamab treatment dose	- The 1.5 mg/kg treatment dose of teclistamab must have been administered ≥ 2 days after Step-up dose 2 - Thereafter, there must have been ≥ 5 days between 1.5 mg/kg treatment doses (or between a 1.5 mg/kg and the switch to the 3 mg/kg dose) and ≥ 12 days between 3 mg/kg treatment doses - On C1D8 (or the first time daratumumab and teclistamab were administered on the same day, in the case of dose delays), teclistamab should have been administered ≥ 3 hours after daratumumab SC. If the patient experienced treatment delays of daratumumab SC longer than 3 months, the interval between daratumumab SC and teclistamab should have been at least 3 hours for the first resumed daratumumab dose				1.5 mg/kg	1.5 mg/kg	1.5 mg/kg	1.5 mg/kg	3 mg/kg *	3 mg/kg *	3 mg/kg [†]
Daratumumab SC (1800 mg)	There must have been ≥ 5 days between doses of daratumumab SC	X			X	X	X	X	X	X	X

SC denotes subcutaneous, C cycle, D day.

*Patients who were receiving 1.5 mg/kg teclistamab weekly treatment doses beyond cycle 3 day 1 when Amendment #2 was implemented were advised to switch to 3 mg/kg teclistamab SC every 2-week dosing on day 1 of the next planned cycle.

[†]Patients who were receiving 3 mg/kg teclistamab every 2-week treatment doses beyond cycle 7 when Amendment #3 was implemented switched to 3 mg/kg teclistamab SC every 4-week dosing on day 1 of the next planned cycle.

Table S2. Summary of Dose Schedule for Investigator's Choice (DPd/DVd) Group.

DPd Treatment Group								
		Treatment Phase (28-day Cycle)						
Study Drug	Notes	Cycles 1-2				Cycles 3-6		Cycle 7+
		Day				Day		Day
		1	8	15	22	1	15	1
Daratumumab SC (1800 mg)	There must have been ≥5 days between each dose of daratumumab SC	X	X	X	X	X	X	X
Pomalidomide (4 mg)	There must have been ≥7 days between the courses of pomalidomide for each cycle	Days 1-21					Days 1-21	Days 1-21
Dexamethasone oral/IV (40 mg or equivalent)	- Dexamethasone also served as a pretreatment medication and was administered ~1-3 hours (±15 minutes) prior to daratumumab SC - For patients >75 years of age or with a BMI of <18.5, dexamethasone may have been administered at a dose of 20 mg	Weekly (days 1, 8, 15, and 22)						
DVd Treatment Group								
		Treatment Phase (21-day Cycle)						
Study Drug	Notes	Cycles 1-8					Cycle 9+	
		Day					Day	
		1	4	8	11	15	1	
Daratumumab SC (1800 mg)	There must have been ≥5 days between each dose of daratumumab SC	X		Cycles 1-3		Cycles 1-3	X	
Bortezomib SC (1.3 mg/m²)	There must have been ≥3 days between doses of bortezomib	X	X	X	X			
Dexamethasone oral/IV (20 mg or equivalent)	- When dexamethasone served as a pretreatment medication, it was to be administered ~1-3 hours (±15 minutes) prior to daratumumab SC - Patients should have received a total of 80 mg dexamethasone weekly in the first 2 out of 3 weeks for the first 8 cycles. 80 mg was divided approximately evenly (~20 mg on each of the 4 days shown per week) except for patients >75 years of age	Cycle 1-8: days 1, 2, 4, 5, 8, 9, 11, and 12 Note: dexamethasone was also considered as premedication on the following daratumumab dosing days: days 1 and 8 in cycles 1-3, day 1 in cycles 4-8						

	or with a BMI of <18.5. These patients should have received 20 mg dexamethasone weekly only on days 1 and 8 for the first 8 cycles		
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DPd denotes daratumumab in combination with pomalidomide and dexamethasone, DVd daratumumab in combination with bortezomib and dexamethasone, SC subcutaneous, IV intravenous, BMI body mass index.

Table S3. Comprehensive Overview of Patient Demographics, Diseases Characteristics, and Prior Lines of Therapy (Intent-to-Treat Population).*

Characteristic	Tec-Dara (N=291)	DPd/DVd (N=296)	Total (N=587)
Age			
Median (range), yr	64 (36-88)	63 (25-84)	64 (25-88)
≥75 yr, no. (%)	31 (10.7)	25 (8.4)	56 (9.5)
Sex, no. (%)			
Male	156 (53.6)	169 (57.1)	325 (55.4)
Female	135 (46.4)	127 (42.9)	262 (44.6)
Race, [†] no. (%)			
White	190 (65.3)	194 (65.5)	384 (65.4)
Asian	68 (23.4)	63 (21.3)	131 (22.3)
Black	13 (4.5)	20 (6.8)	33 (5.6)
Other [‡]	20 (6.9)	19 (6.4)	39 (6.6)
Ethnicity, [†] no. (%)			
Hispanic or Latino	34 (11.7)	47 (15.9)	81 (13.8)
Not Hispanic or Latino	235 (80.8)	235 (79.4)	470 (80.1)
Other [§]	22 (7.6)	14 (4.7)	36 (6.1)
Region of enrollment, no. (%)			
Asia/Pacific	64 (22.0)	61 (20.6)	125 (21.3)
Europe	176 (60.5)	158 (53.4)	334 (56.9)
North America	35 (12.0)	46 (15.5)	81 (13.8)
United States	21 (7.2)	24 (8.1)	45 (7.7)
Canada	14 (4.8)	22 (7.4)	36 (6.1)
South America	16 (5.5)	31 (10.5)	47 (8.0)
ECOG performance status score, [¶] no. (%)			
0	167 (57.4)	160 (54.1)	327 (55.7)
1	108 (37.1)	127 (42.9)	235 (40.0)
2	16 (5.5)	9 (3.0)	25 (4.3)
Bone marrow plasma cells ≥60%, [#] no./total no. (%)	28/286 (9.8)	24/293 (8.2)	52/579 (9.0)
Presence of soft-tissue plasmacytomas, no. (%)			
Yes	41 (14.1)	41 (13.9)	82 (14.0)
Patients with extramedullary plasmacytomas	14 (4.8)	17 (5.7)	31 (5.3)

Patients with paraspinal plasmacytomas	32 (11.0)	31 (10.5)	63 (10.7)
International Staging System stage,** no. (%)			
I	182 (62.5)	185 (62.5)	367 (62.5)
II	85 (29.2)	88 (29.7)	173 (29.5)
III	24 (8.2)	23 (7.8)	47 (8.0)
Prior lines of therapy, no. (%)			
Median, range	2 (1-3)	2 (1-3)	2 (1-3)
1 prior line	108 (37.1)	114 (38.5)	222 (37.8)
2 prior lines	134 (46.0)	134 (45.3)	268 (45.7)
3 prior lines	49 (16.8)	48 (16.2)	97 (16.5)
Previous therapy exposure, no. (%)			
Prior proteasome inhibitor	290 (99.7)	296 (100)	586 (99.8)
Prior immunomodulatory drug	291 (100)	296 (100)	587 (100)
Prior anti-CD38 antibody	15 (5.2)	16 (5.4)	31 (5.3)
Prior transplantation, no. (%)	210 (72.2)	226 (76.4)	436 (74.3)
Autologous	210 (72.2)	225 (76.0)	435 (74.1)
1	154 (52.9)	170 (57.4)	324 (55.2)
≥2	56 (19.2)	55 (18.6)	111 (18.9)
Allogeneic	2 (0.7)	2 (0.7)	4 (0.7)
Refractory status, no. (%)			
Any proteasome inhibitor	117 (40.2)	104 (35.1)	221 (37.6)
Any immunomodulatory drug	247 (84.9)	253 (85.5)	500 (85.2)
Lenalidomide	240 (82.5)	251 (84.8)	491 (83.6)
Double (proteasome inhibitor and immunomodulatory drug)	99 (34.0)	88 (29.7)	187 (31.9)
Median time from diagnosis of multiple myeloma to randomization, yr (range)	3.7 (0.4-20.3)	3.9 (0.2-22.3)	3.8 (0.2-22.3)
Cytogenetic risk ^{††}			
No.	285	294	579
Standard risk, no. (%)	126 (44.2)	145 (49.3)	271 (46.8)
High risk, no. (%)	104 (36.5)	104 (35.4)	20 (3.5)
del(17p)	61 (21.4)	62 (21.1)	123 (21.2)
t(4;14)	47 (16.5)	55 (18.7)	102 (17.6)
t(14;16)	13 (4.6)	4 (1.4)	17 (2.9)
Undetermined, no. (%)	55 (19.3)	45 (15.3)	100 (17.3)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*Patients in the Tec-Dara group were randomized to receive teclistamab combined with daratumumab. Patients in the DPd/DVd group were randomized to receive daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, per investigator's choice. The intent-to-treat population included all patients who underwent randomization.

†Race and ethnicity were self-reported by patients.

‡“Other” includes Native Hawaiian or Pacific Islander (Tec-Dara, n=1 [0.3%]; DPd/DVd, n=0; total, n=1 [0.2%]), American Indian or Alaska Native (0; n=1 [0.3%]; n=1 [0.2%]), not reported (n=14 [4.8%]; n=16 [5.4%]; n=30 [5.1%]), and unknown (n=5 [1.7%]; n=2 [0.7%]; n=7 [1.2%]).

§“Other” includes not reported (Tec-Dara, n=19 [6.5%]; DPd/DVd, n=14 [4.7%]; total, n=33 [5.6%]) and unknown (n=3 [1.0%]; n=0; n=3 [0.5%]).

¶Eastern Cooperative Oncology Group (ECOG) performance status scores range from 0 to 5, with higher scores indicating greater disability.

#Maximum value from bone marrow biopsy or bone marrow aspirate was selected if both results were available.

‡A patient may have had both extramedullary and paraspinal plasmacytomas. Percentages were calculated with the number of patients with ≥ 1 soft-tissue plasmacytoma in each treatment group as the denominator.

**International Staging System is derived based on serum beta-2-microglobulin and albumin.

††Cytogenetic risk abnormalities are based on central fluorescence in situ hybridization testing, or local fluorescence in situ hybridization testing or karyotype testing if central fluorescence in situ hybridization testing is not available. High cytogenetic risk was defined as the presence of ≥ 1 of the following abnormalities: del(17p), t(4;14), or t(14;16).

Table S4. Representativeness of Study Participants.

Category	Example
Disease under investigation	Relapsed refractory multiple myeloma
Special considerations related to	
Sex and gender	Multiple myeloma affects a higher proportion of men compared with women. Per SEER statistics (United States), multiple myeloma is ~1.5× more common in men than women (8.8 versus 6.1 cases per 100,000 persons, respectively). ⁶ Similarly, in a global trends analysis, the age-standardized incidence rate for multiple myeloma, while increasing in both males and females over time, remains higher for males versus females (5.73 vs. 4.37 per 100,000, respectively). ⁷
Age	Per SEER statistics, in the United States, myeloma is most frequently diagnosed among those aged 65-74 years, with a median age at diagnosis of 69 years. ⁶ In contrast, in an analysis of myeloma across Europe, median age was slightly older, ranging from 70 years of age in Italy to 74 years of age in France. ⁸
Race or ethnicity	Within the United States, non-Hispanic Blacks have a higher incidence of multiple myeloma in comparison with other racial groups, regardless of sex. ⁶ In an analysis of multiple myeloma across European countries (France, Germany, Italy, Spain, United Kingdom), multiple myeloma was primarily reported by White patients, ranging from 84-99% of patients. ⁸
Geographical region	Per a global trend analysis, in 2021, age-standardized incidence rates for multiple myeloma were lowest in countries from Oceania, Central Sub-Saharan Africa, Western Sub-Saharan Africa, Central Asia, Southeast Asia, East Asia, and South Asia, and highest in Western Europe and Australasia. In regard to age-standardized mortality rates, rates were the highest in Western Europe, high-income North America, and Australia. ⁷
Overall representativeness of the MajesTEC-3 study	Consistent with the study protocol, the enrolled population was comprised of patients with relapsed/refractory multiple myeloma who had received 1 to 3 prior lines of therapy, including a proteasome inhibitor and lenalidomide (lenalidomide-refractory if only 1 prior line of therapy). In terms of age, race, and sex, the enrolled patients were generally representative of the relapsed/refractory multiple myeloma patient population, with a median age of 64 (range, 25-88), a sex distribution of 55.4%/44.6% for

	men/women, respectively, and 65.4% White patients. Although the overall proportion of Black patients was low (5.6%), the proportion of Black patients enrolled from the United States was high and respective of the United States myeloma population (48.9%; Table S3). Of note, a notable percentage of Asian patients were enrolled (22.3%). While the proportion of the study population with anti-CD38 exposure was low (5.3% overall), substantial treatment effects for anti-CD38-exposed patients were still observed.
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Table S5. Summary of Relative Dose Intensity (Safety Population).*

	Tec-Dara (N=283)	DPd/DVd	
		DPd (N=263)	DVd (N=27)
Teclistamab relative dose intensity ^{†‡}			
No.	281	-	-
Median, % (range)	97.1 (1.2-113.4)	-	-
Daratumumab relative dose intensity [‡]			
No.	283	263	27
Median, % (range)	97.8 (25.0-100.0)	95.5 (20.0-100.0)	90.0 (44.4-100.0)
Pomalidomide relative dose intensity [‡]			
No.	-	263	-
Median, % (range)	-	74.9 (14.3-102.4)	-
Bortezomib relative dose intensity [‡]			
No.	-	-	27
Median, % (range)	-	-	79.5 (50.7-101.0)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population included all patients who received study treatment.

[†]Includes step-up doses as well as repeat step-up doses if applicable.

[‡]Relative dose intensity is calculated considering planned dose per protocol.

Table S6. Summary of Treatment Cycle Delays, Incidence, and Reason for Study**Treatment Dose Modification (Safety Population).***

No. (%)	Tec-Dara (N=283)	DPd/DVd (N=290)
Patients with cycle delays [†]	248 (87.6)	186 (64.1)
Reason for cycle delay		
Adverse event	237 (83.7)	170 (58.6)
Other	124 (43.8)	45 (15.5)
Patients with dose delays and reason		
Patients who received teclistamab ^{†, ‡}	281 (99.3)	-
Patients with teclistamab delay [§]	92 (32.7)	-
Adverse event	60 (21.4)	-
Other	41 (14.6)	-
Patients who received daratumumab [†]	283 (100.0)	290 (100.0)
Patients with daratumumab delay [§]	46 (16.3)	21 (7.2)
Adverse event	36 (12.7)	19 (6.6)
Other	11 (3.9)	4 (1.4)
Patients with dose skipped and reason		
Patients who received teclistamab ^{†, ‡}	281 (99.3)	-
Patients with teclistamab skipped [§]	187 (66.5)	-
Adverse event	172 (61.2)	-
Other	69 (24.6)	-
Patients who received daratumumab [†]	283 (100.0)	290 (100.0)
Patients with daratumumab skipped [§]	174 (61.5)	214 (73.8)
Adverse event	167 (59.0)	210 (72.4)
Other	19 (6.7)	15 (5.2)
Patients with dose reduced and reason		
Patients who received teclistamab ^{†, ‡}	281 (99.3)	-
Patients with teclistamab reduced ^{§, ¶}	80 (28.5)	-
Adverse event	80 (28.5)	-
Other	0	-

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population included all patients who received study treatment.

[†]Percentages are calculated with the number of subjects in each treatment group as denominator.

[‡]Includes patients who received at least one dose of teclistamab.

[§]Percentages are calculated with the number of patients who received the corresponding drug as denominator.

[¶]Dose reduction refers to reduction of protocol-defined cumulative teclistamab dose per cycle, including reducing the teclistamab dose from 3 mg/kg to 1.5 mg/kg and decreasing the dosing frequency. Repeat step-up doses 0.06 mg/kg and 0.3 mg/kg are excluded for dose reduced.

Table S7. Summary of Treatment Response and Minimal Residual Disease Negativity with Odds Ratios (Intent-to-Treat Population).

Variable	Tec-Dara (N=291)	DPd/DVd (N=296)	Odds Ratio (95% CI)*
Overall response, no. (%) [†]	259 (89.0)	223 (75.3)	2.65 (1.68-4.18)
Response, no. (%) [‡]			
Stringent complete response	225 (77.3)	69 (23.3)	
Complete response	13 (4.5)	26 (8.8)	
Very good partial response	14 (4.8)	74 (25.0)	
Partial response	7 (2.4)	54 (18.2)	
Minimal response	1 (0.3)	13 (4.4)	
Stable disease	13 (4.5)	46 (15.5)	
Progressive disease	3 (1.0)	5 (1.7)	
Not evaluable	15 (5.2)	9 (3.0)	
Complete response or better	238 (81.8)	95 (32.1)	9.56 (6.47-14.14)
Very good partial response or better	252 (86.6)	169 (57.1)	4.85 (3.21-7.32)
Median duration of response, months (95% CI)	NE (NE-NE)	23.5 (19.8-29.9)	
Median time to first response, months (range)	1.2 (0.9-25.0)	1.2 (0.7-6.3)	
Median time to complete response or better, months (range)	6.9 (1.0-34.5)	6.9 (1.5-18.8)	
Minimal residual disease next-generation sequencing analysis set, total no. ^{§¶}	262	269	
Minimal residual disease negative (10 ⁻⁵), no. (%)	153 (58.4)	46 (17.1)	6.78 (4.53-10.15)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, CI confidence interval.

*Listed are the Mantel-Haenszel estimates of the common odds ratio for stratified values. Note that an odds ratio of >1 indicates an advantage for Tec-Dara. Confidence interval widths have not been adjusted for multiplicity and may not be used in place of hypothesis testing.

†Overall response is defined as a partial response or better.

‡Response and disease progression were assessed by an Independent Review Committee in accordance with the International Myeloma Working Group response criteria.¹⁻³

§Minimal residual disease negativity was evaluated in the next-generation sequencing primary analysis set, defined as all randomized patients in the study except those recruited in China (due to China instead utilizing next-generation flow cytometry for assessment of patients enrolled at the time the study was initiated).

¶Minimal residual disease was assessed in bone marrow aspirates by next-generation sequencing (clonoSEQ; Adaptive Biotechnologies) in accordance with International Myeloma Working Group guidelines.⁹

Table S8. Summary of Additional Minimal Residual Disease Negativity Analyses.*

Variable	Tec-Dara (N=291)	DPd/DVd (N=296)	Odds Ratio (95% CI)[†]
Minimal residual disease next-generation sequencing analysis set, total no. ^{‡§}	262	269	
Minimal residual disease-negative (10^{-5}) complete response or better, no. (%)	151 (57.6)	46 (17.1)	6.54 (4.37-9.77)
Minimal residual disease-negative (10^{-6}) complete response or better, no. (%)	141 (53.8)	28 (10.4)	9.70 (6.11-15.42)
Sustained (≥ 6 months) Minimal residual disease-negative (10^{-5}) complete response or better, [¶] no. (%)	122 (46.6)	30 (11.2)	6.78 (4.31-10.64)
Sustained (≥ 6 months) Minimal residual disease-negative (10^{-6}) complete response or better, [¶] no. (%)	103 (39.3)	17 (6.3)	9.20 (5.31-15.95)
Minimal residual disease next-generation sequencing-evaluable set ^{§#}			
Minimal residual disease-negative (10^{-5}) complete response or better, no./total no. (%)	150/168 (89.3)	46/73 (63.0)	5.01 (2.51-9.98)
Minimal residual disease-negative (10^{-6}) complete response or better, no./total no. (%)	140/160 (87.5)	28/67 (41.8)	9.61 (4.81-19.20)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, CI confidence interval.

*Minimal residual disease-negative complete response or better rate, at both 10^{-5} and 10^{-6} thresholds were prespecified sensitivity analyses as outlined in the statistical analysis plan. All other analyses were post-hoc.

[†]Listed are the Mantel-Haenszel estimates of the common odds ratio for stratified values. Note that an odds ratio of >1 indicates an advantage for Tec-Dara. Confidence interval widths have not been adjusted for multiplicity and may not be used in place of hypothesis testing.

‡Minimal residual disease negativity was evaluated in the next-generation sequencing primary analysis set, defined as all randomized patients in the study except those recruited in China (due to China instead utilizing next-generation flow cytometry for assessment of patients enrolled at the time the study was initiated).

§Minimal residual disease was assessed in bone marrow aspirates by next-generation sequencing (clonoSEQ; Adaptive Biotechnologies) in accordance with International Myeloma Working Group guidelines.⁹

¶Sustained minimal residual disease-negative complete response or better was defined as minimal residual disease-negative complete response or better achieved and confirmed ≥ 6 months apart (-1 month) without a positive minimal residual disease assessment, progressive disease, or subsequent antineoplastic therapy in between.

#Minimal residual disease next-generation sequencing-evaluable set (post-hoc) defined as patients who achieved a complete response or better, have a successful baseline calibration, and had ≥ 1 post-baseline minimal residual disease sample with a positive or negative result (per next-generation sequencing) at the indicated testing threshold.

Table S9. Summary of Overall Survival Analyses (Intent-to-Treat Population).

	Tec-Dara (N=291)	DPd/DVd (N=296)
Cox model*		
Hazard ratio (95% CI)	0.46 (0.32-0.65)	
Restricted means survival time, [†] months		
Estimate (95% CI)	30.30 (29.19-31.42)	28.15 (26.98-29.31)
Difference (95% CI)	2.15 (0.54-3.77)	

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, CI confidence interval.

*Hazard ratio and 95% confidence interval from a prespecified Cox proportional hazards model with treatment as the sole explanatory variable, and stratified with International Staging System (I vs. II or III) and number of prior lines (1 vs. 2 or 3), as randomized. A hazard ratio <1 indicates an advantage for Tec-Dara. The crossing of the Kaplan-Meier curves suggests a departure from the proportional hazards assumption for overall survival. As such, a prespecified analysis of restricted mean survival time was performed.

[†]Restricted mean survival time was measured by the area under the Kaplan-Meier overall survival curve up to 34.1 months, the smaller value of the longest overall survival event time observed from either arm at the clinical cutoff. A difference of >0 indicates an advantage (ie, longer overall survival time on average) for Tec-Dara arm.

Note: Confidence interval widths have not been adjusted for multiplicity and may not be used in place of hypothesis testing.

Table S10. Time to Worsening in MySIm-Q per Total Symptoms Subscale.

	Tec-Dara (N=291)	DPd/DVd (N=296)
Time to worsening in MySIm-Q total symptom subscale		
Number of events, %	52 (17.9)	66 (22.3)
Number of censored, %	239 (82.1)	230 (77.7)
Kaplan-Meier estimate, months		
25% quantile, 95% CI	36.0 (32.7-39.5)	19.5 (14.0-26.6)
Median, 95% CI	NE (39.5-NE)	39.9 (36.3-NE)
75% quantile, 95% CI	NE (NE-NE)	NE (39.9-NE)
P value*	0.0002	
Hazard ratio [†]	0.50 (0.34-0.72)	
6-month event-free rate, % (95% CI)	95.9 (92.6-97.8)	91.2 (86.8-94.2)
12-month event-free rate, % (95% CI)	94.7 (91.0-96.9)	82.3 (76.4-86.8)
18-month event-free rate, % (95% CI)	91.2 (86.8-94.2)	76.2 (69.5-81.6)
24-month event-free rate, % (95% CI)	90.3 (85.8-93.5)	74.1 (67.2-79.8)
30-month event-free rate, % (95% CI)	84.5 (79.1-88.7)	67.0 (59.3-73.7)
36-month event-free rate, % (95% CI)	75.6 (68.1-81.6)	63.3 (54.7-70.6)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib, MySIm-Q Multiple Myeloma Symptom and Impact Questionnaire, CI confidence interval, NE not evaluable.

*P value is based on a stratified log-rank test stratified with International Staging System (I vs. II or III) and number of prior lines (1 vs. 2 or 3), as randomized.

[†]Hazard ratio and 95% CI from a Cox proportional hazards model with treatment as the sole explanatory variable and stratified with International Staging System staging (I vs. II or III) and

number of prior lines (1 vs. 2 or 3), as randomized. A hazard ratio <1 indicates an advantage for Tec-Dara.

Note: The anchor-based worsening minimally importance difference on the MySIm-Q Symptom Scale was the mean change on the MySIm-Q Symptom Scale associated with a 2-point increase on the Patient Global Impression of Severity symptom question between baseline and cycle 4 day 1. Worsening was defined as a score greater than or equal to the minimally importance difference threshold without subsequent improvement to a score below this level.

Note: Confidence interval widths have not been adjusted for multiplicity and may not be used in place of hypothesis testing.

Table S11. Summary of Most Common* Serious Treatment-Emergent Adverse Events (Safety Population).[†]

No. (%)	Tec-Dara (N=283)	DPd/DVd (N=290)
Any treatment-emergent serious adverse event	200 (70.7)	181 (62.4)
Pneumonia	47 (16.6)	38 (13.1)
COVID-19 pneumonia	32 (11.3)	8 (2.8)
Cytokine release syndrome	27 (9.5)	0
COVID-19	19 (6.7)	6 (2.1)
Pyrexia	14 (4.9)	5 (1.7)
Febrile neutropenia	13 (4.6)	14 (4.8)
Pneumonia bacterial	9 (3.2)	4 (1.4)
Upper respiratory tract infection	9 (3.2)	7 (2.4)
Influenza	6 (2.1)	11 (3.8)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*Most common defined as $\geq 3\%$ of patients in any treatment group.

[†]The safety population for this analysis included all patients who received study treatment.

Table S12. Summary of Deaths and Cause of Death (Safety Population).*

No. (%)	Tec-Dara (N=283)	DPd/DVd (N=290)
Total number of patients who died during study	45 (15.9)	96 (33.1)
Primary cause of death		
Adverse event	20 (7.1)	17 (5.9)
Adverse event of COVID-19	3 (1.1)	0
Disease progression	13 (4.6)	59 (20.3)
Other	12 (4.2)	20 (6.9)
Total number of patients who died within 30 days of last study treatment dose	10 (3.5)	15 (5.2)
Primary cause of death		
Adverse event	8 (2.8)	12 (4.1)
Adverse event of COVID-19	0	0
Disease progression	2 (0.7)	2 (0.7)
Other	0	1 (0.3)
Total number of patients who died within 60 days of first study treatment dose	6 (2.1)	4 (1.4)
Primary cause of death		
Adverse event	4 (1.4)	3 (1.0)
Adverse event of COVID-19	0	0
Disease progression	2 (0.7)	1 (0.3)
Other	0	0

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population included all patients who received study treatment.

Table S13. Summary of Deaths due to Treatment-Emergent Adverse Events (Safety Population).*

Event, no. (%)	Tec-Dara (N=283)		DPd/DVd (N=290)	
	Death due to TEAE	Death due to TEAE that was related [†]	Death due to TEAE	Death due to TEAE that was related [†]
Number of patients overall	20 (7.1)	12 (4.2)	17 (5.9)	5 (1.7)
Treatment-emergent adverse event by preferred term [‡]				
Pneumonia	3 (1.1)	3 (1.1)	1 (0.3)	1 (0.3)
COVID-19	2 (0.7)	1 (0.4)	0	0
Sepsis	2 (0.7)	2 (0.7)	1 (0.3)	0
Septic shock	2 (0.7)	1 (0.4)	1 (0.3)	0
Respiratory failure	2 (0.7)	2 (0.7)	0	0
Sudden death	1 (0.4)	1 (0.4)	3 (1.0)	0
COVID-19 pneumonia	1 (0.4)	1 (0.4)	0	0
Enterococcal sepsis	1 (0.4)	0	0	0
Enterovirus myocarditis	1 (0.4)	1 (0.4)	0	0
Pseudomonas infection	1 (0.4)	0	0	0
West Nile viral infection	1 (0.4)	0	0	0
Multiple organ dysfunction syndrome	1 (0.4)	0	0	0
Acute myocardial infarction	1 (0.4)	0	0	0
Hepatitis fulminant	1 (0.4)	1 (0.4)	0	0
Haemophagocytic lymphohistiocytosis	1 (0.4)	1 (0.4)	0	0
New onset refractory status epilepticus	1 (0.4)	0	0	0
Pneumonia pneumococcal	0	0	1 (0.3)	1 (0.3)
Pseudomonal sepsis	0	0	1 (0.3)	1 (0.3)
General physical health deterioration	0	0	1 (0.3)	0
Acute respiratory distress syndrome	0	0	1 (0.3)	0
Pneumothorax	0	0	1 (0.3)	0
Pulmonary embolism	0	0	1 (0.3)	1 (0.3)
Cardiac failure	0	0	1 (0.3)	0
Hepatic failure	0	0	1 (0.3)	1 (0.3)

Cerebrovascular accident	0	0	1 (0.3)	0
Cognitive disorder	0	0	1 (0.3)	0
Duodenal ulcer perforation	0	0	1 (0.3)	0
Myelodysplastic syndrome	0	0	1 (0.3)	1 (0.3)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population included all patients who received study treatment.

†Relationship to study agent was assessed by the investigator, and includes adverse events that were related to ≥ 1 of the components of study treatment: teclistamab, daratumumab, pomalidomide, bortezomib, dexamethasone.

‡Some patients may have had ≥ 1 treatment-emergent adverse event at the time of death

Table S14. Summary of Treatment-emergent Cytokine Release Syndrome and Supportive Treatments (Safety Population).*

	Tec-Dara (N=283)
Number of patients with cytokine release syndrome, no. (%)	170 (60.1)
Maximum toxicity grade, no. (%)	
Grade 1	125 (44.2)
Grade 2	45 (15.9)
Grade 3	0
Grade 4	0
Grade 5	0
Cytokine release syndrome with outcome death, [†] no. (%)	0
Number of patients with cytokine release syndrome leading to discontinuation of teclistamab, [‡] no. (%)	0
Number of patients with cytokine release syndrome leading to discontinuation of all study treatment, [§] no. (%)	0
Number of patients with multiple cytokine release syndrome events, no. (%)	66 (23.3)
Number of patients with multiple grade 2 cytokine release syndrome events	5 (1.8)
Occurrence of cytokine release syndrome, [¶] no. (%)	
Step-up dose 1	96 (33.9)
Step-up dose 2	85 (30.0)
1 st treatment dose	50 (17.7)
Repeat step-up dose schedule [#]	7 (2.5)
Cycle 1	167 (59.0)
Cycle 2	10 (3.5)
Cycle 3+	5 (1.8)
Median time from last treatment dose to new onset of cytokine release syndrome, days (range)	2 (1-9)
Median duration of cytokine release syndrome, days (range)	2 (1-22)
Outcome of cytokine release syndrome	
Recovered or resolved, no. (%)	275 (100)
Number of patients with supportive measures to treat cytokine release syndrome,** no. (%)	161 (56.9)

Anti-IL6 receptor tocilizumab	83 (29.3)
Multiple doses at any time during study for >1 cytokine release syndrome event	11 (3.9)
>1 dose for a single cytokine release syndrome event	5 (1.8)
Corticosteroids	8 (2.8)
Intravenous fluids	29 (10.2)
Vasopressor used	1 (0.4)
Single ^{††}	1 (0.4)
Multiple	0
Oxygen used	13 (4.6)
Nasal cannula low flow (≤ 6 l/min)	13 (4.6)
Paracetamol	120 (42.4)
Antibiotics	57 (20.1)
Other	32 (11.3)

Tec-Dara denotes teclistamab in combination with daratumumab.

*The safety population for this analysis included all patients in the Tec-Dara group who received study treatment.

[†]Cytokine release syndrome with outcome assessed as “fatal” on the adverse event electronic clinical report form page.

[‡]Cytokine release syndrome with action taken to teclistamab as drug withdrawn on the adverse event electronic clinical report form page.

[§]Includes those patients indicated as having discontinued all study treatment due to adverse events on the end of treatment clinical report form page.

[¶]Patients may appear in >1 category. Occurrence is based on the last treatment visit on or prior to the day in which the cytokine release syndrome occurred. The same event for a patient may be reported twice, in a treatment dose category and a cycle category.

[#]Repeat step-up dose schedule refers to any required repeat step-up dose and the 1.5 mg/kg dose (if applicable) immediately following any required repeat step-up dose. Any 1.5 mg/kg dose that

is immediately following a required repeat step-up dose and is not the first treatment dose will be considered a repeat treatment dose.

^lTotal number of cytokine release syndrome events (n=275) used as the denominator.

****Supportive measures to treat cytokine release syndrome and cytokine release syndrome symptoms are included.**

^{††}Vasopressor use was reported in 1 patient with grade 1 cytokine release syndrome: the patient developed fever and diarrhea, which were considered symptoms of cytokine release syndrome. The diarrhea was managed with racecadotril and was summarized as vasopressor use.

Table S15. Summary of Occurrence of Cytokine Release Syndrome by Toxicity Grade (Safety Population).*

	Tec-Dara (N=283)					
	Toxicity Grade					
	Total	1	2	3	4	5
Total number of patients with treatment-emergent cytokine release syndrome, no. (%)	170 (60.1)	135 (47.7)	45 (15.9)	0	0	0
Occurrence of cytokine release syndrome [†]						
Step-up dose 1	96 (33.9)	73 (25.8)	23 (8.1)	0	0	0
Step-up dose 2	85 (30.0)	68 (24.0)	17 (6.0)	0	0	0
1 st treatment dose	50 (17.7)	42 (14.8)	8 (2.8)	0	0	0
Repeat step-up dose schedule [‡]	7 (2.5)	7 (2.5)	0	0	0	0
Cycle 1	167 (59.0)	130 (45.9)	45 (15.9)	0	0	0
Cycle 2	10 (3.5)	10 (3.5)	0	0	0	0
Cycle 3+	5 (1.8)	5 (1.8)	0	0	0	0

Tec-Dara denotes teclistamab in combination with daratumumab.

*The safety population for this analysis included all patients in the Tec-Dara group who received study treatment.

[†]Patients may appear in more than one category. Occurrence is based on the last treatment visit on or prior to the day in which the CRS event occurred. The same event for a patient may be reported twice, in a treatment dose category and a cycle category.

‡Repeat step-up dose schedule refers to any required repeat step-up dose and the 1.5 mg/kg dose (if applicable) immediately following any required repeat step-up dose. Any 1.5 mg/kg dose that is immediately following a required repeat step-up dose and is not the first treatment dose will be considered a repeat treatment dose.

Table S16. Summary of Immune Effector Cell-Associated Neurotoxicity Syndrome Events (Safety Population).*

	Tec-Dara (N=283)
Number of patients with immune effector cell-associated neurotoxicity syndrome, no. (%)	3 (1.1)
Maximum toxicity grade, no. (%)	
Grade 1	2 (0.7)
Grade 2	0
Grade 3	0
Grade 4	1 (0.4)
Grade 5	0
Immune effector cell-associated neurotoxicity syndrome with outcome death, [†] no. (%)	0
Number of patients with immune effector cell-associated neurotoxicity syndrome leading to discontinuation of teclistamab, [‡] no. (%)	1 (0.4)
Number of patients with immune effector cell-associated neurotoxicity syndrome leading to discontinuation of all study treatment, [§] no. (%)	0
Number of patients with multiple immune effector cell-associated neurotoxicity syndrome events, no. (%)	0
Occurrence of immune effector cell-associated neurotoxicity syndrome, [¶] no. (%)	
Step-up dose 1	1 (0.4)
Step-up dose 2	0
Repeat step-up dose schedule [#]	0
Cycle 1	3 (1.1)
Cycle 2	0
Cycle 3+	0
Median time from last treatment dose to new onset of immune effector cell-associated neurotoxicity syndrome, days (range)	2 (1-3)
Outcome of immune effector cell-associated neurotoxicity syndrome [‡]	

Recovered or resolved, no. (%)	3 (100)
Concurrent cytokine release syndrome ^{l**}	
Yes, no. (%)	1 (33.3)
No, no. (%)	2 (66.7)

Tec-Dara denotes teclistamab in combination with daratumumab.

*The safety population for this analysis included all patients in the Tec-Dara group who received study treatment.

[†]Immune effector cell-associated neurotoxicity syndrome with outcome assessed as “fatal” on the adverse event electronic clinical report form page.

[‡]Immune effector cell-associated neurotoxicity syndrome with action taken to teclistamab as drug withdrawn on the adverse event electronic clinical report form page.

[§]Includes those patients indicated as having discontinued all study treatment due to immune effector cell-associated neurotoxicity syndrome on the end of treatment clinical report form page.

[¶]Patients may appear in >1 category. Occurrence is based on the last treatment visit on or prior to the day in which the immune effector cell-associated neurotoxicity syndrome occurred. The same event for a patient may be reported twice, in a treatment dose category and a cycle category.

[#]Repeat step-up dose schedule refers to any required repeat step-up dose and the 1.5 mg/kg dose (if applicable) immediately following any required repeat step-up dose. Any 1.5 mg/kg dose that is immediately following a required repeat step-up dose and is not the first treatment dose will be considered a repeat treatment dose.

^lTotal number of immune effector cell-associated neurotoxicity syndrome events (n=3) used as denominator.

^{**}Concurrent cytokine release syndrome considers immune effector cell-associated neurotoxicity syndrome events that occur during or within 7 days of the end date of cytokine release syndrome.

Table S17. Summary of Treatment-emergent Grade ≥ 3 Cytopenic Adverse Events Over Time (Safety Population).*

	Event Onset Time						
	Total	≤ 6 Months	$>6-\leq 12$ Months	$>12-\leq 18$ Months	$>18-\leq 24$ Months	$>24-\leq 30$ Months	>30 Months
Tec-Dara							
Total number of patients within window, n [†]	283	283	244	233	218	210	196
Patients with ≥ 1 grade ≥ 3 treatment-emergent cytopenic event, [‡] no. (%)	233 (82.3)	219 (77.4)	95 (38.9)	54 (23.2)	48 (22.0)	41 (19.5)	24 (12.2)
MedDRA preferred term, no. (%)							
Neutropenia [§]	214 (75.6)	193 (68.2)	84 (34.4)	49 (21.0)	44 (20.2)	38 (18.1)	21 (10.7)
Neutropenia	214 (75.6)	192 (67.8)	83 (34.0)	49 (21.0)	44 (20.2)	37 (17.6)	21 (10.7)
Febrile neutropenia	21 (7.4)	13 (4.6)	3 (1.2)	2 (0.9)	2 (0.9)	2 (1.0)	0
Neutropenic infection	2 (0.7)	2 (0.7)	0	0	0	0	0
Anemia [§]	58 (20.5)	49 (17.3)	6 (2.5)	2 (0.9)	1 (0.5)	1 (0.5)	2 (1.0)
Thrombocytopenia [§]	55 (19.4)	52 (18.4)	5 (2.0)	0	2 (0.9)	3 (1.4)	0
Lymphopenia [§]	59 (20.8)	54 (19.1)	7 (2.9)	3 (1.3)	6 (2.8)	3 (1.4)	5 (2.6)
DPd/DVd							
Total number of patients within window, no. [†]	290	290	228	175	138	117	91
Patients with ≥ 1 grade ≥ 3 treatment-emergent cytopenic event, [‡] no. (%)	252 (86.9)	248 (85.5)	62 (27.2)	53 (30.3)	37 (26.8)	29 (24.8)	20 (22.0)
MedDRA preferred term, no. (%)							
Neutropenia [§]	229 (79.0)	225 (77.6)	57 (25.0)	45 (25.7)	34 (24.6)	27 (23.1)	19 (20.9)
Neutropenia	228 (78.6)	224 (77.2)	57 (25.0)	45 (25.7)	33 (23.9)	25 (21.4)	19 (20.9)
Febrile neutropenia	17 (5.9)	8 (2.8)	4 (1.8)	0	1 (0.7)	3 (2.6)	1 (1.1)

Neutropenic sepsis	2 (0.7)	1 (0.3)	0	0	0	0	1 (1.1)
Anemia [§]	50 (17.2)	41 (14.1)	6 (2.6)	5 (2.9)	2 (1.4)	2 (1.7)	1 (1.1)
Thrombocytopenia [§]	68 (23.4)	57 (19.7)	6 (2.6)	7 (4.0)	2 (1.4)	1 (0.9)	1 (1.1)
Lymphopenia [§]	32 (11.0)	31 (10.7)	3 (1.3)	6 (3.4)	4 (2.9)	3 (2.6)	1 (1.1)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population for this analysis included all patients who received study treatment.

[†]Includes patients who are in the treatment-emergent adverse event reporting period for the specific window.

[‡]Percentages calculated with the number of patients within each window as denominator.

[§]Preferred term grouping.

Noting that patients were counted only once in a window for any given event, regardless of the number of times they experienced the event within the specific time window.

Noting that adverse events are coded using MedDRA version 28.0.

Table S18. Summary of Most Common* Treatment-emergent Infections and Infestations (Safety Population).[†]

No. (%)	Tec-Dara (N=283)		DPd/DVd (N=290)	
	Any Grade	Grade 3/4	Any Grade	Grade 3/4
Any infection	273 (96.5)	153 (54.1)	244 (84.1)	126 (43.4)
Most common treatment-emergent infection and infestation [‡]				
COVID-19	124 (43.8)	17 (6.0)	97 (33.4)	6 (2.1)
Upper respiratory tract infection	115 (40.6)	12 (4.2)	88 (30.3)	7 (2.4)
Pneumonia	65 (23.0)	47 (16.6)	53 (18.3)	43 (14.8)
Nasopharyngitis	62 (21.9)	0	57 (19.7)	0
Sinusitis	52 (18.4)	5 (1.8)	17 (5.9)	3 (1.0)
Rhinovirus infection	44 (15.5)	5 (1.8)	10 (3.4)	1 (0.3)
Bronchitis	40 (14.1)	2 (0.7)	31 (10.7)	6 (2.1)
Influenza	38 (13.4)	8 (2.8)	43 (14.8)	10 (3.4)
COVID-19 pneumonia	34 (12.0)	32 (11.3)	12 (4.1)	7 (2.4)
Urinary tract infection	29 (10.2)	4 (1.4)	27 (9.3)	1 (0.3)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*Most common treatment-emergent infection of any grade defined as occurring in $\geq 10\%$ of patients in either treatment group; shown with percent occurrence of respective grade 3/4 infection.

[†]The safety population for this analysis included all patients who received study treatment.

[‡]Per preferred term.

Table S19. Summary of Treatment-emergent Grade ≥ 3 Infections and Infestations Over Time (Safety Population).*

	Event Onset Time						
	Total	≤ 6 Months	$>6-\leq 12$ Months	$>12-\leq 18$ Months	$>18-\leq 24$ Months	$>24-\leq 30$ Months	>30 Months
Tec-Dara							
Infections and infestations							
Total number of patients within window [†]	283	283	244	233	219	210	197
Patients with ≥ 1 grade ≥ 3 treatment-emergent infections and infestations, [‡] no. (%)	153 (54.1)	100 (35.3)	41 (16.8)	25 (10.7)	24 (11.0)	24 (11.4)	16 (8.1)
DPd/DVd							
Infections and infestations							
Total number of patients within window [†]	290	290	228	175	138	117	91
Patients with ≥ 1 grade ≥ 3 treatment-emergent infections and infestations, [‡] no. (%)	126 (43.4)	60 (20.7)	39 (17.1)	28 (16.0)	14 (10.1)	9 (7.7)	8 (8.8)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*The safety population for this analysis included all patients who received study treatment.

[†]Includes patients who are in the treatment-emergent adverse event reporting period for the specific window.

[‡]Percentages calculated with the number of patients within each window as denominator.

Noting that patients were counted only once in a window for any given event, regardless of the number of times they actually experienced the event within the specific time window.

Noting that adverse events are coded using MedDRA version 28.0.

Table S20. Summary of Second Primary Malignancies* (Safety Population).[†]

No. (%)	Tec-Dara (N=283)	DPd/DVd (N=290)
Patients with 1 or more secondary primary malignancy	35 (12.4)	25 (8.6)
Type, preferred term		
Cutaneous/noninvasive	23 (8.1)	13 (4.5)
Basal cell carcinoma	14 (4.9)	7 (2.4)
Squamous cell carcinoma of skin	14 (4.9)	7 (2.4)
Bowen's disease	4 (1.4)	1 (0.3)
Lip squamous cell carcinoma	1 (0.4)	0
Penile squamous cell carcinoma	1 (0.4)	0
Skin squamous cell carcinoma recurrent	1 (0.4)	0
Hematologic malignancies	8 (2.8)	4 (1.4)
Acute myeloid leukemia	4 (1.4)	1 (0.3)
Myelodysplastic syndrome	2 (0.7)	1 (0.3)
Histiocytic sarcoma	1 (0.4)	0
Hodgkin's disease	1 (0.4)	0
Myelofibrosis	1 (0.4)	0
Acute lymphocytic leukemia	0	1 (0.3)
Marginal zone lymphoma	0	1 (0.3)
Solid malignancy	6 (2.1)	10 (3.4)
Malignant melanoma	2 (0.7)	0
Adenocarcinoma gastric	1 (0.4)	0
Adenocarcinoma of colon	1 (0.4)	1 (0.3)
Gastric cancer	1 (0.4)	0
Renal cancer	1 (0.4)	0
Renal cell carcinoma	1 (0.4)	0
Bladder cancer	0	1 (0.3)
Colon cancer	0	1 (0.3)
Intestinal adenocarcinoma	0	1 (0.3)
Invasive ductal breast carcinoma	0	2 (0.7)
Laryngeal squamous cell carcinoma	0	1 (0.3)
Metastatic malignant melanoma	0	1 (0.3)
Prostate cancer	0	1 (0.3)
Squamous cell carcinoma of lung	0	1 (0.3)
Thyroid cancer	0	1 (0.3)

Tec-Dara denotes teclistamab in combination with daratumumab, DPd/DVd daratumumab in combination with dexamethasone and either pomalidomide or bortezomib.

*Includes all recurrent prior diagnosed malignancies and second primary malignancies reported during the study.

†The safety population for this analysis included all patients who received study treatment.

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